A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP

Gastroenterologist & Endoscopist

PN 421926PK

FOCUS GASTROENTEROLOGY

27 May 2026

Dr Amanda Richards
Pinnacle Surgery

Dear Dr Richards,

Re: Mr Richard Healey
DOB: 16/07/1956 Phone: 0419 991 904
Address: 2 Sahara Court, PORTSEA, VIC, 3944
Email: richard@healeyandco.com.au

Summary

- **Gastroscopy findings of minor duodenal ulceration and distal oesophagitis.**
- **Gastric biopsies negative for Helicobacter pylori and malignancy.**
- **Clinical suspicion of a pharyngeal pouch; investigation deferred at present.**
- **Education provided regarding NSAID use and gastric protection.**
- **No action required by you.**

I had the pleasure of reviewing Richard in the clinic following his gastroscopy. You will be pleased to know that the biopsies taken during the procedure have not shown any unusual cells or evidence of malignancy.

The gastroscopy did identify some minor duodenal ulcers. While we excluded Helicobacter pylori on histological grounds, there was evidence of inflammation within the stomach and distal oesophagitis. The latter finding is consistent with Richard's long-standing history of reflux. Regarding the duodenal ulcers, Richard noted that he uses Nurofen approximately once a week for knee pain, occasionally increasing this frequency. I suspect this intermittent NSAID use is the likely cause of these small ulcers.

During the procedure, I noted some resistance at the level of the upper oesophageal sphincter. While a pharyngeal pouch can be easily missed during a standard gastroscopy as it folds in on itself, this resistance is often a clinical clue to its presence. We discussed the fact that such a pouch can lead to mucus pooling and could potentially contribute to his chronic cough.

Impression and Plan

Richard is currently participating in a clinical trial for his respiratory symptoms, which he reports has been transformative. We had a pragmatic discussion regarding further investigation of a possible pharyngeal pouch via a barium swallow. At this stage, Richard is keen to avoid any new formal diagnoses that might complicate his standing within the clinical trial or require reporting to the TGA, given how well he is currently faring.

I am comfortable with a 'wait and watch' approach. I have advised Richard to transition to paracetamol for analgesia where possible. I have provided him with a script for pantoprazole, with instructions to use this as gastric protection only if he requires a protracted course of NSAIDs in the future. We will hold off on the barium swallow for now. I plan to touch base with Richard for a review in six to nine months' time.

All appointments and enquires

Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Peter Meggyesy

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